

Q I'm carrying twins. Will my prenatal care be different from someone carrying a single baby?

You'll be more closely monitored than a woman carrying just one baby. You won't necessarily have complications, but since there is higher risk with multiple pregnancies you will have more prenatal appointments and ultrasounds than for a single pregnancy. Your checkups may be with a maternal-fetal medicine specialist in a separate office, rather than solely at your OB/GYN's office. The specialist can give you support and advice about multiple births.

Q I wanted to have a home birth. Can I still do that if I'm carrying twins?

Multiple births generally happen at the hospital because there is a greater risk of complications during labor, and thus they need larger medical teams. It's more likely that your babies will be in a difficult position for birth or have a cord prolapse (when the umbilical cord starts to protrude from your uterus ahead of the baby). It's also more likely that one or both of your babies will have a low birth weight, which requires special care in a hospital immediately after birth.

Q I feel a bit daunted about having twins. Is there someone I can talk to?

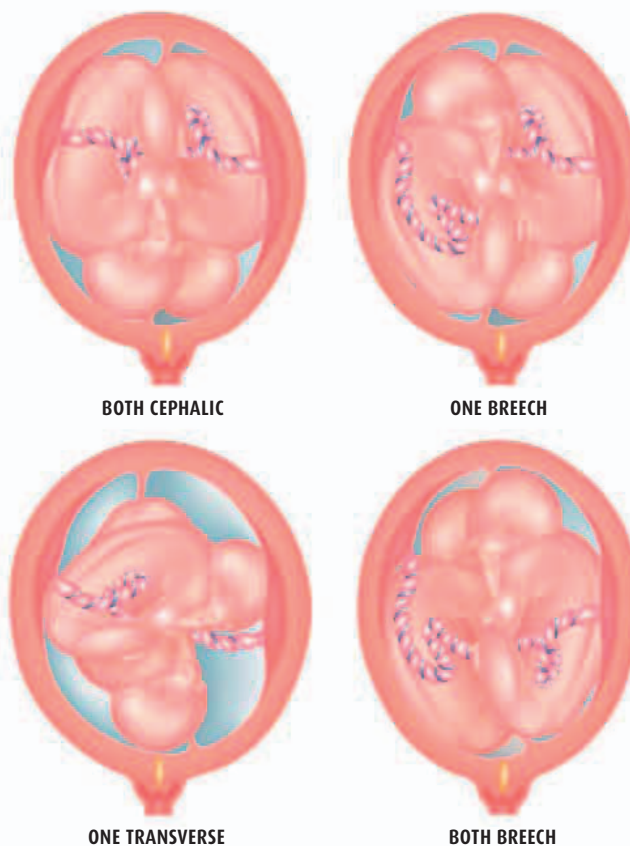
Your doctor can put you in touch with local twin (and multiple) support groups and other recent mothers of multiples. Try to gather as much information and reassurance as you can—and also any practical tips about how to best take care of yourself and your babies in pregnancy and after the birth. Organizations such as the National Organization of Mothers of Twins Clubs (see pp.338–39) are a good source of information and advice for parents of twins. You should also be able to start childbirth classes earlier than women carrying singletons, usually around 24 weeks, rather than the usual 30 or 32 weeks. Encourage your partner to come to these classes, too—although the role of a partner can never be underestimated for any mother and baby, when it comes to multiple births, the practical and emotional support of your partner will be more significant than ever.

Q Is it possible to have a vaginal birth if I'm carrying multiples?

If you are having three or more babies, a cesarean section will be recommended as the safest option. With twins, a vaginal birth is possible if your pregnancy is uncomplicated and the babies are in good positions at labor.

Toward the end of pregnancy, multiple babies have less space in the uterus and the placenta(s) can become less efficient. For these reasons, unless the babies arrive prematurely, you'll have a scheduled early C-section for triplets or more; twin pregnancies are offered induction at 37 weeks plus for monochorionic twins, and 38 weeks plus for dichorionic twins (see opposite and pp.224–25). You could be offered an elective C-section for a twin

pregnancy if there are known complications such as a shared placenta or amniotic sac, one or both of your babies are especially small, the placenta is low-lying, you have had a previous C-section, or if you've had complications such as preeclampsia. If your pregnancy has been a healthy one, you may be able to have a natural birth. Your options will depend on how the twin who is closest to the birth canal (the "presenting" twin) is positioned in your uterus.



Both cephalic (head down)

When both babies are head down, there is a good chance of successful vaginal birth for both. Around 45 percent of twins present in this position.

One breech (bottom first)

About 25 percent of twins present with the first baby head down, and the second breech. In this case, vaginal birth of the first twin is possible. If the presenting twin is breech, a cesarean section is advised.

One transverse (horizontal)

Babies who lie transverse can turn, so if the presenting twin is head down, vaginal birth may be possible. If the presenting baby is breech, a cesarean section is more likely.

Both breech When both babies are bottom first, in most cases you will be offered a cesarean section because the babies are less likely to turn.

Women pregnant with multiples are at a slightly increased risk of preeclampsia, anemia, low-lying placenta, and midterm bleeding. For all these reasons, your doctor will be extra vigilant throughout your pregnancy.